

Controlled Substance Tracking

Standard:

To establish minimum requirements for controlled substance ordering, storage, handling, supplying, accounting, and documentation.

Purpose:

The purpose of this standard is to ensure accurate, timely, and complete reporting of the use, expiration, and custody of controlled substances carried by credentialed providers in the EMS system.

Application:

Each system agency that utilizes controlled substances must implement a tracking system that is approved by the Chief Medical Officer. This system must be able to be audited on a regular basis, as required by law, or more, at the request of the Chief Medical Officer.

Receipt of Shipment

Receipt of controlled substances from a shipper will require verification of the shipment by two individuals prior to adding to that agencies stock. Once the shipment is verified, it should be added to the tracking system.

Storage

Controlled substances will be stored in an area with a minimum of two locking mechanisms, at least one of which may be a combination lock and one of which may be keyed (this may include a vehicle lock). While on a shift, in the "pouch," controlled substances must be stored either in an area behind two locking mechanisms or on the person of a provider credentialed at the PL-5 level or higher.

Verification

Master Inventory Safe

The safe must be audited at the beginning of each week by two approved individuals and documented within the narcotics tracking system. No controlled substances may be issued while the tracking system is being audited. Any time the safe is opened, existing counts must be verified as well. If the count cannot be reconciled, the OCMO must be notified.

Custody change:

At shift change, or at other times, narcotics may need to change hands. Both providers must acknowledge which specific narcotics are changing hands by referencing vial numbers. Providers must document the date and time of this transaction in the narcotics tracking system. In the event of a discrepancy, the OCMO must be notified.

Self-Audits:

Providers not subject to shift change must audit their narcotics with a witness every two weeks. This must be logged in the tracker. In the event of a discrepancy, the OCMO must be notified.

Administration:

Whenever narcotics are administered, they must be documented both on the patient care report and in the narcotics tracker. The narcotics tracker must reference the incident or run number. An indication must be documented, the amount administered, and the amount wasted, if any, must be written in the tracker. The amount administered must match on both the tracker and the patient care report. If any narcotics are wasted, a witness must witness the waste and sign. In

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order of preference, witnesses to waste should be: another credentialed system provider, a receiving nurse or physician, a TCOLE-certified law enforcement officer.